

NISARGA

A TRAUMA-INFORMED GUIDE FOR BODYWORK & BREATHWORK STUDENTS

The Issue Lives in the Tissue

Trauma Release Through Conscious Touch and Breath

NISARGA ERYK DOBOSZ

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WELCOME

A Letter Before We Begin



Dear student, dear fellow traveller on this path,

When I first began working with breath in 1998, in a small yoga school in Warsaw, I had no idea that I was beginning a lifelong conversation with the human nervous system. Years later, in my sessions and trainings across Europe, Asia and the Americas, I kept meeting the same quiet truth: the issue lives in the tissue. What we could not feel at the time does not simply disappear. Very often, the body holds it for us, in the breath that never quite reaches the belly, in the jaw that will not soften, in the shoulders that still brace against a world that once felt unsafe.

This ebook is my gift to you as a bodywork and breathwork student. It brings together what I have learned over more than twenty years with hands and breath, what my teachers have given me, and what careful research can and cannot yet tell us about trauma. I have tried to be honest about both. You will find sentences such as "research suggests" and "many practitioners observe" throughout. That is not hesitation. It is respect: for science, for our clients, and for the mystery of the body.

Trauma work asks a lot of us. It asks for skill, but even more it asks for presence, patience and humility. The good news is that you do not have to be perfect. You have to be safe, kind, curious and willing to learn. If you can offer that, the body will often show you the way. The body whispers. Our job is to slow down enough to hear it.

Take your time with these pages. Try the practices on yourself first. Notice what moves in you. And remember: you cannot teach others something you are not.

With heartfelt compassion and dedication,

Nisarga Eryk Dobosz

BEFORE YOU START

How to Use This Ebook

This ebook is written for students of bodywork and breathwork: massage and myofascial therapists, breathwork facilitators, yoga and movement teachers, and anyone learning to work with touch and breath in a trauma-informed way. It is a companion to live training, not a replacement for it. Trauma-informed skills are learned in the body, with supervision, feedback and practice.

The ebook has three parts. **Part One** explains what trauma is, how it may live in the body and the fascia, and how the nervous system responds to threat. **Part Two** opens the two doorways I work with every day, trauma-informed touch and the breath, and then shows how to combine them in a session. **Part Three** covers safety, consent and scope of practice, what to do when a client is overwhelmed, how to support release and integration, and how to care for yourself.

Along the way you will find:

- **Try this** boxes: short practices to explore on yourself first, then with practice partners.
- **Reflection** boxes: questions for your journal or for discussion with your study group.
- **Key takeaways**: the essentials of each chapter on one card.
- **Science corner** boxes: what research supports, what is still debated, and where we are using metaphor.
- **Safety** boxes: when to slow down, stop, or refer.

SAFETY

Safety at a glance

This ebook is educational. It is not medical or psychological advice, and it does not qualify you to treat trauma, PTSD or any mental health condition.

Bodywork and breathwork are not psychotherapy. Work within your training and your local legal scope of practice, and refer clients to qualified health and mental health professionals when needed (see Chapter 8).

If reading these pages stirs strong feelings in you, pause. Feel your feet, look around the room, breathe out slowly. Reach out to a trusted person or a qualified therapist if you need support.

In an emergency, or if someone is at risk of harming themselves or others, contact local emergency services (112 in Poland and across the EU).

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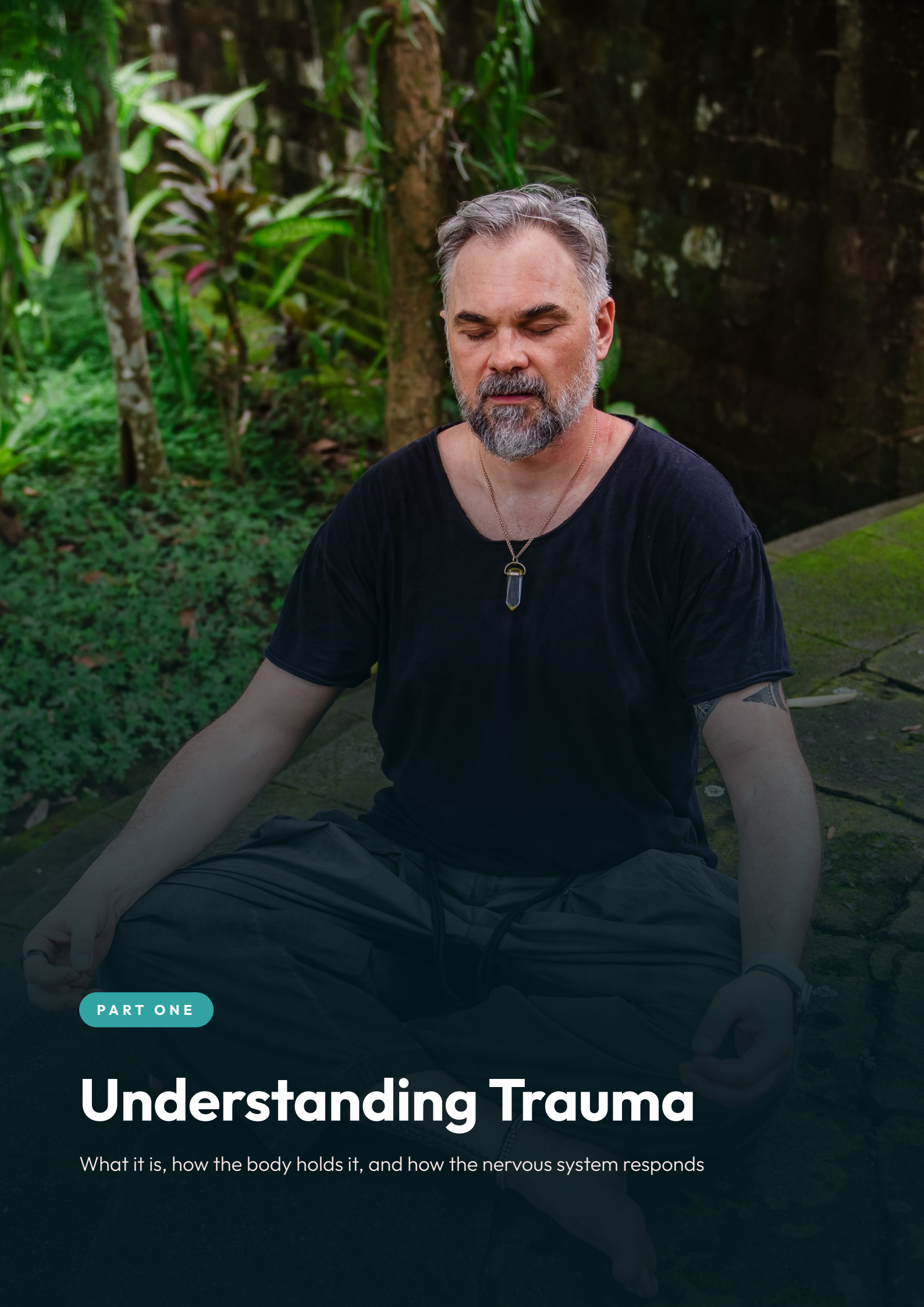
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PART ONE

Understanding Trauma

What it is, how the body holds it, and how the nervous system responds



CHAPTER 1

What Trauma Really Is

From stress to overwhelm, and why the response matters more than the event

> Trauma is not only what happened to us. It is what happened inside us, and what could not happen at the time.

The word trauma comes from the Greek for "wound". In everyday language we use it for very different experiences: a car accident, a violent attack, a painful divorce, a childhood in which no one really saw us. As bodywork and breathwork students, we need a definition that is clear enough to keep us safe and humble, and broad enough to honour the many ways people are hurt.

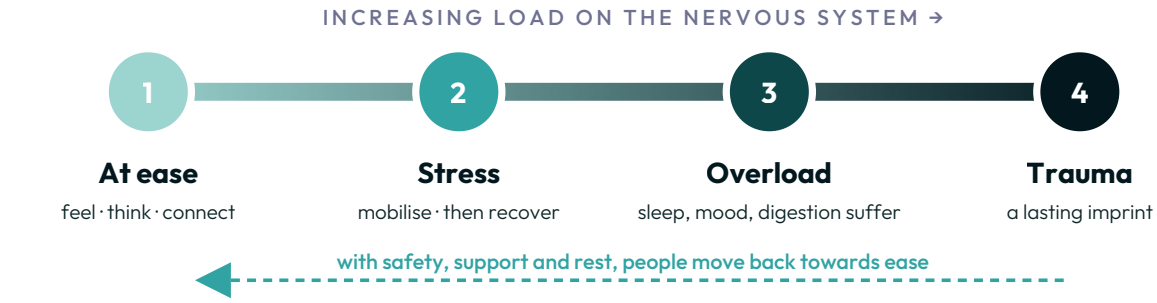
A working definition

A widely used definition from the US mental health agency SAMHSA describes trauma in three parts, sometimes called the three Es: an **event** or set of circumstances, the way it is **experienced** as harmful or threatening, and its lasting **effects** on a person's functioning and well-being. I love this definition because it puts the experience and the effects at the centre. Two people can live through the same event; one recovers within weeks, while the other carries it in their body for years.

This is why I often say that trauma is not only the event. It is the way our system was overwhelmed, and what it had to do to survive.

The stress to trauma continuum

Stress is part of life. A healthy nervous system can rise to a challenge and then come back to rest. Problems begin when the challenge is too big, too fast, too long, or when we are alone with it. Then the system may not return to baseline.



From everyday stress to trauma: a simplified continuum. Real lives move back and forth along it.

You can picture it as a line. At one end, we are **at ease**: we can feel, think and connect. Then we meet **stress**: we tense, we mobilise, and we recover. When stress is intense or unrelenting, we may become **overloaded**: sleep, mood and digestion begin to suffer. And when an experience overwhelms our capacity to cope, especially if we feel helpless or alone, it may leave a lasting imprint: **trauma**. Movement along this line goes both ways. With support, rest and safety, people move back towards ease.

Many faces of trauma

There is no single kind of trauma. In my trainings I invite students to recognise a wide spectrum, without turning it into a diagnosis:

- **Acute trauma**: a single overwhelming event, such as an accident, an assault, a sudden loss or a medical emergency.
- **Chronic trauma**: repeated or prolonged experiences, such as ongoing abuse, domestic violence, bullying or living in danger.
- **Complex and developmental trauma**: repeated interpersonal harm or neglect, often in childhood, when the nervous system is still forming and the people who should protect us are the source of fear.
- **Relational and emotional trauma**: experiences of betrayal, humiliation, abandonment, or simply not being seen, held or allowed to feel.
- **Physical and medical trauma**: injuries, surgeries, painful procedures, difficult birth experiences.
- **Sexual trauma**: any unwanted sexual experience. This is especially important for bodyworkers, because touch itself can be a reminder.

- **Collective, cultural and identity-based trauma**: war, displacement, disaster, discrimination and oppression that affect whole communities and can echo through generations.
- **Vicarious trauma**: the impact of repeatedly witnessing or listening to the suffering of others. This one is about us as practitioners, and we will return to it in Chapter 11.

Some people also speak of **existential** or **spiritual** wounding: the collapse of meaning, faith or belonging. These are not clinical categories, but they are very real in the room.

Many therapists speak of "big T" trauma (events like violence or disaster) and "small t" trauma (the subtle, repeated hurts of not being met). These are informal terms, not diagnoses. I find them useful for one reason: they remind us not to compare suffering. What matters is not how dramatic the story sounds, but how much the nervous system had to carry.

> **Do not compare suffering. Ask instead: how much did this nervous system have to carry, and was anyone there to help carry it?**

How common is trauma?

Very common. In the World Mental Health Surveys, which interviewed nearly 69,000 adults in 24 countries, about 70% reported at least one traumatic event in their lifetime. Yet most people who live through a traumatic event do not develop post-traumatic stress disorder (PTSD). Across 26 population surveys, the lifetime prevalence of PTSD was about 4% of all adults, and between 5% and 6% of people exposed to trauma. Figures vary a lot between countries and groups.

This matters for two reasons. First, many of the people on your table or mat will carry some trauma history, often unspoken. Second, human beings are remarkably resilient. Trauma is common, and so is recovery.

PTSD and complex PTSD

PTSD is a mental health condition that can develop after frightening or distressing events. The NHS describes its main symptom as distressing memories of the event that keep returning, as flashbacks, intrusive images or nightmares. Other symptoms can include avoiding reminders, feeling disconnected from yourself and the world, difficulty controlling emotions or feeling numb, irritability, sleep problems, constant watchfulness for danger, and unexplained aches and pains. **Complex PTSD** can develop after repeated or prolonged trauma from which escape is difficult, such as child abuse or neglect or ongoing domestic abuse. The World Health Organization's ICD-11 describes it as PTSD together with lasting difficulties in regulating emotions, a deeply negative sense of self, and difficulties in relationships.

These are diagnoses made by qualified health professionals, and they are treated with evidence-based therapies such as trauma-focused cognitive behavioural therapy and EMDR. As bodyworkers and breathworkers we never diagnose. But we can recognise signs, work gently, and support clients in getting the right help.

Childhood experiences and lifelong health

One of the most influential pieces of research in this field is the Adverse Childhood Experiences (ACE) study. In a sample of more than 9,000 adults in the United States, researchers found that the more types of adversity people had experienced in childhood (such as abuse or household dysfunction), the higher their risk of many later health problems, including heart disease, lung disease, depression and substance misuse. This is a statistical association across a population, not a prediction for any one person. But it confirms what many of us feel in our hands: early experience shapes the body.

Trauma is a response, not a weakness

Here is the shift that changed everything for me. When we are threatened, our nervous system prepares to fight, to flee or, when neither is possible, to freeze. These are not choices. They are ancient, intelligent survival programmes. Peter Levine, the founder of Somatic Experiencing, describes trauma as linked to survival responses that could not be completed: energy mobilised for action that was never discharged. This is a therapeutic model rather than a proven mechanism, and research on it is still developing. But as a way of meeting clients, I find it profoundly kind. Nothing is wrong with you. Your body did what it needed to do to survive.

REFLECTION

- ? Which forms of trauma in the list above surprised you, or touched something in you?
- ? How do you usually respond under stress: do you speed up, push, freeze, or try to please?
- ? What helps you come back to ease after a hard day?

KEY TAKEAWAYS

- ✓ Trauma is defined less by the event than by the experience of overwhelm and its lasting effects.
- ✓ Trauma is common; PTSD is less common; recovery is possible and frequent.
- ✓ We never diagnose. We recognise, respect, work gently and refer.
- ✓ Survival responses are intelligent, not shameful.



CHAPTER 2

How Trauma Lives in the Body and the Fascia

The issue lives in the tissue: what we observe, what research shows, and where we use metaphor

> The body had been carrying it the whole time.

Have you ever wondered why a knot in the shoulder returns no matter how many massages someone gets? Why, after a heartbreak, the chest feels heavy and tight? Why, years after a frightening event, the jaw still clenches when someone raises their voice?

In more than two decades of sessions I have seen it again and again. When we slow down and meet the tissue with patience, old emotions can come to the surface: tears, a deep exhale, trembling, sometimes laughter. Afterwards people often look at me and say, "I didn't know I was carrying that." This is what I mean when I say **the issue lives in the tissue**.

CLIENT STORY

What are you carrying on your shoulders?

A woman came to me with a frozen shoulder. She could barely lift her arm, and months of treatment hadn't changed much. Before we began any touch work, I asked her a simple question: "What are you carrying on your shoulders?"

She paused. Then the tears came, slowly at first, then all at once. In that moment she realised how much responsibility she had carried for decades: for her family, her friends, for everyone around her. She had always been the strong one, the one who held it all together, and she had never once put it down.

We stayed with that for a while and let the feelings move through her. Then we checked her shoulder again, before any hands-on work at all. Her range of movement had improved by about eighty percent.

Nothing had touched the joint. What changed was that something long held finally had a voice. For me it was a clear reminder that emotions don't just live in the mind. They settle into our tissues, and when they are felt and expressed, the body can let go.

Fascia: the living web

Fascia is the connective tissue that wraps and connects every muscle, organ, nerve and bone. Healthy fascia is hydrated and adaptable; its layers can glide over each other, and it lets us move with ease. It is also far more than packing material.

SCIENCE CORNER **what research shows about fascia**

Fascia is richly innervated. A 2022 systematic review of histological studies found nerve fibres in many fascial tissues, including fibres associated with pain signalling and fibres of the sympathetic nervous system, our stress system. This supports the idea that fascia takes part in how we sense our body.

Fascia can change its tension. Laboratory research has found contractile cells called myofibroblasts in human fascia, and isolated fascial tissue can slowly contract in response to certain biochemical signals. These contractions are slow, over minutes to hours, not instant.

What is not proven: that specific memories or emotions are "stored" in fascia. There is no direct evidence for this. When I say "the issue lives in the tissue", I am describing a clinical observation and a useful teaching image, not a laboratory finding.

So what might be happening when an emotion seems to live in a body area? Here is my understanding, offered with humility. Under threat, muscles brace to protect us: the shoulders rise, the jaw sets, the belly tightens, the breath shortens. If this bracing is repeated for months or years, it can become a habit of the nervous system, and the tissue adapts to the posture we keep. Because fascia is so densely supplied with sensory and sympathetic nerves, a braced body may keep sending "something is wrong" signals to the brain, and the brain keeps answering "stay on guard". When this area is finally met with safety, warmth and slow attention, the brace can soften, and the feelings that were held alongside it can be felt.

> **Fascia is not your enemy. It has been loyally holding what you could not feel at the time, until you were ready.**

Holding patterns

Over the years I have noticed that certain patterns repeat so often that they work like a personal body map. They are not rules, and every body is unique. But they may help you begin to listen:

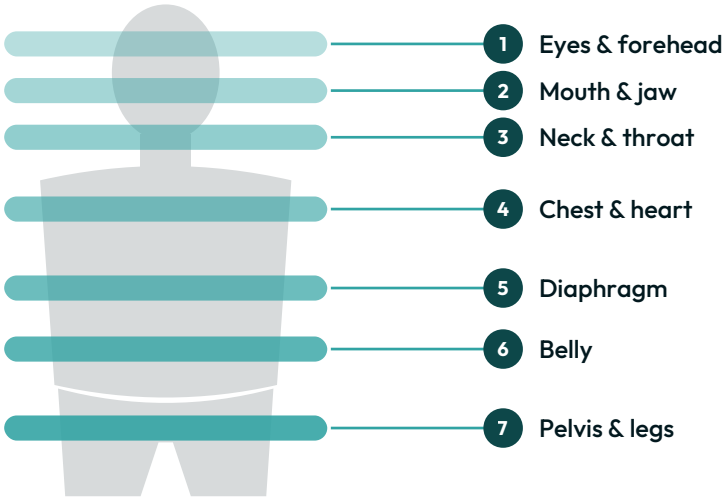
- **Fear and startle** often show in the upper body: raised shoulders, a tight neck, a set jaw, a tense forehead and eyes, the head pulled forward.
- **Anxiety** often lives in the chest and diaphragm: shallow, high breathing and an armoured ribcage.
- **Grief** gathers around the heart and throat: a weight on the sternum, a collapsing chest.
- **Shame** shows in the face and posture: the gaze drops, the body shrinks inward, we want to disappear.

- **Shutdown and depression** often feel like a whole-body heaviness: flat energy, numbness, collapse.

Research supports part of this picture. In a study published in 2014, more than 700 participants from different cultures coloured in body maps showing where they felt different emotions, and the maps were remarkably consistent: anger and fear, for example, were felt strongly in the chest and upper body, while sadness was linked with reduced feeling in the limbs. This tells us how emotions are felt, not that they are stored in particular tissues. Still, it is a beautiful confirmation that emotion is a bodily experience.

Belts of tension

Long before fascia research became popular, the psychoanalyst Wilhelm Reich, a pioneer of body-oriented psychotherapy, described "armouring": chronic muscular tension that he believed held back emotion. He mapped it in horizontal segments, or belts, from the eyes down to the pelvis. Many breathwork and bodywork traditions still use these belts as a map: the eyes and forehead, the mouth and jaw, the neck and throat, the chest, the diaphragm, the belly, and the pelvis. Reich's theories are historical, and many were never scientifically supported, so I use this map as a way of paying attention, not as a diagnosis. In practice, it helps us notice where breath stops and where the body holds.



Seven horizontal belts of tension often used as a map in body-oriented work. A guide for attention, not a diagnosis.

The diaphragm and the deep core

Two places deserve special attention in trauma-informed work. The **diaphragm** is our main breathing muscle, and it sits right at the crossroads of the body. When we are frightened, the breath becomes high and held, and the diaphragm can feel like a locked door between the upper and lower body. The **psoas**, a deep hip flexor connecting the spine to the legs, is often described by bodyworkers as a "fear muscle", because we curl up and draw the legs in when we protect ourselves. This idea is popular, but

there is little direct research behind it. I hold it as a practitioner's observation: when the belly and the deep hip region soften, many clients feel safer and more grounded. Both areas are sensitive. Work there slowly, with clear consent, and never press deeply into the belly of someone who is not ready.

Why emotions surface when tissue softens

When I work with slow, listening touch and the client breathes a little more freely, several things seem to come together. The nervous system, met by another calm nervous system, begins to settle. Bracing patterns can let go. Sensations that were numbed or avoided come back into awareness. And with sensation, feeling returns. That is why tears, sighs, warmth and trembling often appear during or after a session. They are not something we create or force. They are the body's own way of letting go when it feels safe enough.

TRY THIS

Three minutes of body listening

1. Sit or lie down comfortably. Place one hand on your chest and one on your belly.
2. Breathe naturally. Ask silently: "What is living here today?"
3. Notice any tightness, heaviness, warmth or numbness, without trying to change it. There is nothing to fix. Simply witness.
4. After three minutes, feel your feet on the floor and look around the room before you continue.

REFLECTION

- ? Where does your own body tend to brace when you are stressed?
- ? What happens in that area when you place a warm hand there and breathe out slowly?

KEY TAKEAWAYS

- ✓ Fascia is richly innervated, including by the stress system, and can slowly change its tension.
- ✓ "The issue lives in the tissue" is a clinical observation and a teaching image, not proof that memories are stored in fascia.
- ✓ Holding patterns are protective strategies. We meet them with curiosity, never with force.



CHAPTER 3

The Nervous System: Fight, Flight, Freeze and the Window of Tolerance

Understanding the states your clients move through, and the breath as a window onto them

> Every moment, beneath our awareness, the nervous system is asking one question: am I safe?

Your nervous system is constantly scanning, outside and inside. It listens to faces and voices, to sounds and movement, and also to the heartbeat, the breath, the belly and the tone of the tissue. How it answers the question "am I safe?" shapes whether we can digest, sleep, connect, learn and heal.

Two branches

The autonomic nervous system, which runs the body without our conscious control, has two main branches. The **sympathetic** branch is the accelerator. It prepares us for action: the heart speeds up, the breath becomes faster and higher, blood flows to the muscles and digestion pauses. The **parasympathetic** branch is the brake. It supports rest, digestion and repair: the heart slows, the breath deepens and the belly softens. The **vagus nerve** is the main nerve of the parasympathetic system. Interestingly, most of its fibres, often estimated at around 80%, carry information from the body up to the brain. This is one reason why what we do with the body, including breath and touch, can influence how safe we feel.

The defence cascade

Researchers describe a sequence of hard-wired defence responses, sometimes called the defence cascade:

1. **Arousal**: something catches our attention, and we become alert.
2. **Fight or flight**: the body mobilises to defend itself or escape.
3. **Freeze**: high alert with stillness, a fight-or-flight response "on hold", watching for the right moment.
4. **Tonic or collapsed immobility**: when escape seems impossible, the body may go rigid or limp, numb or far away. This is the last resort.
5. **Quiescence**: when the danger is over, rest and healing.

Animals usually return to their normal state when the threat has passed. Humans often do not. We may stay locked in a pattern of hypervigilance or shutdown long after the danger is gone. Many clients carry deep shame about having "frozen" during an assault or accident. One of the most healing things we can say is: freezing is not a choice and not a failure. It is the body trying to keep you alive.

> **“ Freezing is not a failure. It is the body trying to keep you alive.**

A useful map: three states

Many trauma-informed practitioners, including me, use the language of **Polyvagal Theory**, developed by Stephen Porges, to describe three broad states:

- **Safe and social** ("ventral vagal"): calm, present, curious and connected. We can make eye contact, laugh, feel and think.
- **Mobilised** (sympathetic): fight or flight. Racing heart, fast breath, tension, irritation, anxiety, restlessness.
- **Shut down** ("dorsal vagal"): collapse, numbness, heaviness, disconnection, feeling far away.

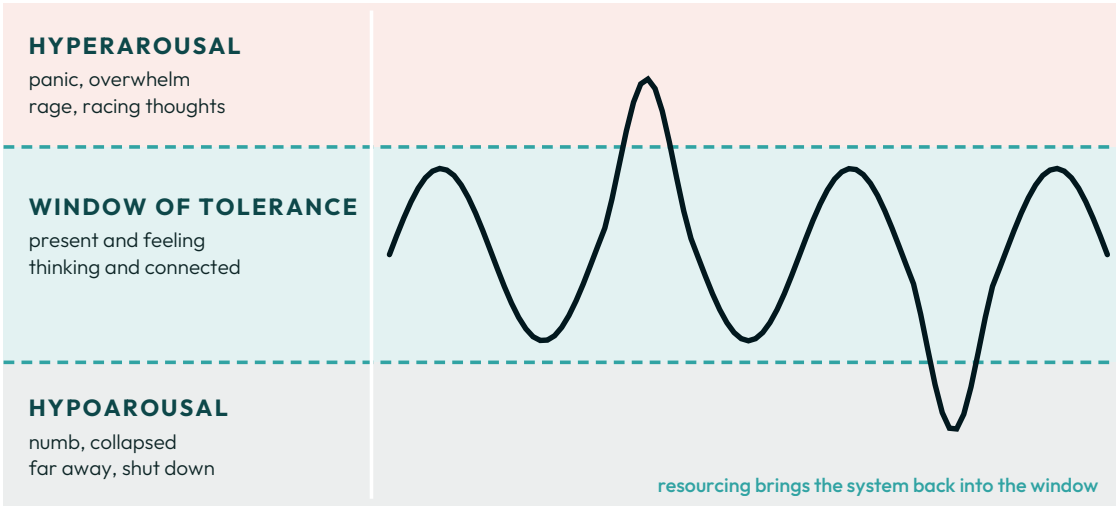
SCIENCE CORNER **a map, not the territory**

Polyvagal Theory has been very influential in trauma-informed practice. Its language of safety, mobilisation and shutdown helps clients make sense of their experience. However, several of its core physiological claims, for example about how the branches of the vagus nerve evolved and how they control the heart, are strongly disputed by other researchers. I use it as a helpful, compassionate map, not as settled science.

You may also hear of the "triune brain": the idea that we have a reptilian, an emotional and a rational brain stacked on top of each other. Neuroscientists now regard this as a myth, because brains did not evolve in neat layers. The simpler and truer message is that under threat, fast survival responses can override slower, reflective thinking.

The window of tolerance

The psychiatrist Daniel Siegel coined the term **window of tolerance** for the range of arousal in which we can feel strong emotions and sensations and still stay present, think and connect. Above the window is **hyperarousal**: overwhelm, panic, anger, racing thoughts. Below it is **hypoarousal**: numbness, collapse, spacing out, feeling far away. Trauma tends to narrow the window, so that small triggers can push a person out of it.



The window of tolerance. Trauma-informed work stays mostly inside the window and gently widens it over time.

Here is the heart of trauma-informed bodywork and breathwork: **we work inside the window, and we help it widen, slowly**. We do not push people above the window for a big release, and we do not leave them below it in a quiet shutdown that looks like relaxation. A healthy nervous system is not one that is always calm. It is one that is flexible: it can mobilise when needed and return to safety afterwards.

The breath as a window onto the nervous system

Breathing is the one function of the autonomic nervous system that is both automatic and open to our conscious influence. That is why it is such a precious doorway. It also tells us, moment by moment, where a person is:

- **Safe and present**: breath is easy, often low and slow, with a natural pause after the exhale.

- **Mobilised**: breath becomes fast and high in the chest, sometimes audible; the client may sigh, gasp, or hold the breath before speaking.
- **Shut down**: breath becomes very shallow and barely visible, or seems to stop; the body is still and the face may go blank.

Learning to watch the breath without staring is one of the first skills I teach. The breath will often tell you that something has changed long before the client does.

TRY THIS

Mapping your own states

1. Over one day, pause three times and ask: "Where am I right now: safe and social, mobilised, or shut down?"
2. Notice your breath in each state: its speed, its depth, and where it moves in your body.
3. Then try one small shift: a longer, slower exhale, looking around the room, or feeling your feet. Notice what changes.

REFLECTION

- ? What pushes you above your window of tolerance? What drops you below it?
- ? What are your three most reliable resources for returning to the window?

KEY TAKEAWAYS

- ✓ Fight, flight, freeze and collapse are automatic survival responses, not choices.
- ✓ The three-state map is useful in practice, but its physiology is debated. Hold it lightly.
- ✓ Trauma-informed work stays inside the window of tolerance and widens it gradually.
- ✓ The breath is a live window onto the nervous system.



CHAPTER 4

Recognising the Signs of Trauma

What may show up in daily life, in the body, and in the session

> The body whispers before it shouts.

Trauma rarely announces itself. Most clients do not arrive saying, "I have trauma." They come with a stiff neck, insomnia, back pain, anxiety, or a feeling that something is "stuck". Recognising possible signs helps us to work more gently. It never means diagnosing, and it never means telling someone what happened to them.

Signs that may appear in daily life

Health services such as the NHS list many common responses after trauma. They include:

- **Emotional and mental**: anxiety, irritability, low mood, guilt or shame, numbness, feeling detached, difficulty concentrating, flashbacks or intrusive memories, avoiding reminders.
- **Physical**: sleep problems and nightmares, being easily startled, muscle tension, headaches, digestive problems, fatigue, persistent pain.
- **Behavioural and relational**: withdrawing from others, difficulty trusting, overworking or over-pleasing, substance misuse, self-harming behaviour.

All of these can have many other causes. That is exactly why we do not interpret. We notice, we stay curious, and we encourage medical advice when symptoms are new, severe or unexplained.

Signs you may notice in a session

On the table or the mat, the nervous system speaks through small cues. Learn to recognise them:

- **Signs of mobilisation**: the breath speeds up or is held; hands clench; the jaw tightens; the eyes dart or the client keeps checking the door; the skin flushes or sweats; a sudden urge to talk, joke or get up.
- **Signs of shutdown**: the breath becomes very shallow; the face goes blank or "glassy"; the voice gets very quiet or the client stops responding; the body feels limp or heavy; the skin goes pale or cool; the client says they feel far away, foggy or "not here".
- **Signs of settling**: a spontaneous deeper breath; a sigh; the eyes soften; swallowing; gurgling in the belly; warmth; the shoulders drop; colour returns to the face.

A useful habit: when you notice a cue, slow down and check in, rather than carrying on with your plan.

Dissociation

Dissociation is a disconnection from thoughts, feelings, memories, the body or surroundings. Everyone dissociates a little, for example when daydreaming. After trauma, dissociation can become a protective strategy: if I cannot escape, I leave inside. The NHS describes symptoms such as feeling disconnected from yourself or the world around you, feeling that things are unreal, and gaps in memory.

In bodywork and breathwork, dissociation can look deceptively like deep relaxation. The client is still and quiet. But there is a difference. In relaxation, the person is present and responsive, and the breath is easy. In dissociation, they are absent, the breath may be very shallow, and they may struggle to answer or not remember part of the session afterwards. Touch to sensitive areas, lying face down, closed eyes, intense breathing and strong music can all make dissociation more likely for some people.

> **“Stillness is not always peace. Learn to tell the difference between a body that has settled and a body that has left.**

TRY THIS **Practising observation with a partner**

1. With a practice partner, take turns lying down for five minutes while the other sits nearby without touching.
2. The observer simply watches the breath, face, hands and skin colour, and notes any changes.
3. Afterwards, compare notes: what did the observer see, and what did the receiver feel? Where did they match, and where not?

REFLECTION

? How do you personally tell the difference between relaxation and "spacing out"?

? Which signs of mobilisation or shutdown do you find hardest to notice in others?

KEY TAKEAWAYS

- ✓ Signs of trauma are non-specific. Notice, stay curious, never diagnose.
- ✓ Watch for mobilisation, shutdown and settling cues, especially in the breath.
- ✓ Dissociation can look like relaxation. Check for presence, not just stillness.



PART TWO

The Two Doorways

Trauma-informed touch, the breath, and how to weave them together



CHAPTER 5

Trauma-Informed Touch

Hands as extensions of the heart: fascia, conscious touch and releasing held tension

> Our hands are extensions of our heart.

From the moment we are born, touch is how we learn whether the world is safe. For many people with a trauma history, touch is complicated: it may have been absent, unpredictable or harmful. This is why touch can be such powerful medicine, and why it must be offered with exceptional care.

SCIENCE CORNER **touch and the nervous system**

Our skin contains special nerve fibres, called C-tactile afferents, that respond best to slow, gentle stroking at roughly 1 to 10 centimetres per second, about the speed of a caress. They are linked to the pleasant, emotional and social side of touch, rather than to detecting texture or pressure.

Reviews of massage research report that massage is associated with reduced anxiety, low mood and pain in many studies, although many of these studies are small and of mixed quality.

What this means for us: slow, warm, predictable touch speaks directly to the body's sense of safety. Speed, depth and surprise can do the opposite.

Principles of trauma-informed touch

Trauma-informed care rests on six principles described by SAMHSA: safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; and attention to cultural, historical and gender issues. In the treatment room, I translate them like this:

- **Safety first**: a warm, private, predictable space; the client knows what will happen and can stop at any time.
- **Transparency**: explain what you are going to do and why, before you do it.
- **Choice and control**: the client decides on clothing, positions, areas, pressure and pace, and can change their mind at any moment.
- **Collaboration**: you are not the expert on their body; they are. You are a skilled, caring companion.
- **Respect for difference**: gender, culture, religion, sexuality and history shape how touch feels. Ask, don't assume.

A practice of conscious touch: the five Ls

Here is how I invite students to approach every contact, especially in trauma-informed sessions.

1. **Land in yourself**: before touching anyone, feel your own feet, breath and heart. Your nervous system is the first instrument in the room.
2. **Let them know**: say where you would like to touch and how, and ask for a clear yes. "I would like to place my hand on your upper back. Is that okay?"
3. **Lightly arrive**: let your hands approach slowly and come into contact gently, with your whole palm, like a bird landing.
4. **Listen**: stay still for a few breaths before doing anything. Feel the temperature, the breath under your hands, the quality of the tissue. Let the tissue tell you how deep and how fast to go.
5. **Leave gently**: when you finish, slow down and leave the body as carefully as you arrived. Many clients feel the departure of hands more than their arrival.

> **“ The tissue will tell you how deep and how fast to go, if you are willing to listen.**

Working with held tension

When we meet held tension, the temptation is to push through it. In trauma-informed work, we do the opposite. We meet the edge of the restriction, wait, and follow. In my experience, slow, sustained pressure, given time, often allows tissue to soften and lets the nervous system decide that it is safe to let go. Some practical guidelines:

- **Slower is deeper**: depth comes from time and presence, not force. Pain usually makes the body brace further.

- **Work at the edge, not beyond it**: ask the client to rate the intensity, and keep it where they can breathe and stay present.
- **Offer resourcing touch**: a still, warm hand on the back of the heart, the shoulder or the feet can be as important as releasing touch. Alternate between the two.
- **Go slowly near vulnerable areas**: the throat, chest, belly, inner thighs, pelvis and face are sensitive for many trauma survivors. Ask specifically, and accept a no without question.
- **Follow, don't fix**: if the body wants to move, stretch or unwind, follow it gently. If it wants stillness, give it stillness.
- **Never interpret tension for the client**: you might sense "grief" in the chest, but it is not your place to name it. Ask, "What do you notice here?"

Reading the body with respect

Observing posture, breath and movement before a session can give you valuable information: where the body braces, where the breath does not reach, how the person stands and holds themselves. I encourage students to practise this kind of body reading, but always as a respectful hypothesis, never as a verdict. Share observations as invitations: "I notice your breath stays high in your chest. Would you like to explore that together?" The client remains the authority on their own experience.

TRY THIS

Resourcing touch with a practice partner

1. Ask your partner where on their body a still, supportive touch would feel good: perhaps the upper back, the shoulders or the feet.
2. Agree on a signal for "stop" and for "more" or "less".
3. Place your whole palm there and do nothing at all for two minutes. Simply breathe and listen.
4. Then leave slowly and ask: "What did you notice?" Swap roles.

REFLECTION

- ? When did you last receive touch that made you feel truly safe? What made it so?
- ? Where in your own touch do you tend to rush, push or "do" too much?

KEY TAKEAWAYS

- ✓ Slow, warm, predictable touch supports safety; speed, depth and surprise can threaten it.
- ✓ Trauma-informed touch is built on safety, transparency, choice, collaboration and respect.
- ✓ Slower is deeper. Work at the edge, alternate releasing and resourcing touch, and treat body reading as a hypothesis, never a diagnosis.



06

CHAPTER 6

The Breath and Trauma

Gentle breath awareness, the safety of breathwork, and clear cautions about intense practices

> Breath is the bridge between what happens to us and what happens within us.

I have been facilitating breathwork for many years, and I still feel awe every time I witness what happens when breath is given space and safety. Breath is automatic, yet we can influence it. It changes with every emotion, and in turn it can change how we feel. That is why it is such a powerful doorway into trauma work, and why it must be used with as much care as touch.

SCIENCE CORNER **breath and the nervous system**

Slow breathing: a systematic review of slow breathing techniques (usually fewer than ten breaths per minute, often around six) found they were associated with higher heart rate variability, a marker of flexible regulation, and with greater relaxation and lower anxiety and arousal.

Breathwork and stress: a 2023 meta-analysis of 12 randomised controlled trials found breathwork was associated with lower self-reported stress than control conditions, with a small-to-medium effect. Most studies had a moderate risk of bias.

Cyclic sighing: in a one-month study of healthy adults, five minutes a day of cyclic sighing (a double inhale and a long exhale) improved mood and lowered resting breathing rate, and on some measures it did better than mindfulness meditation.

PTSD: a 2022 trial with 85 US military veterans found that a structured breathing-based programme (Sudarshan Kriya Yoga) was not inferior to cognitive processing therapy for PTSD symptoms. This is encouraging, but it tested one specific programme, delivered by trained teachers. It does not mean that any breathwork treats PTSD.

Start with awareness, not technique

For trauma-informed work, I always begin with breath **awareness** rather than breath **control**. For many trauma survivors, being told how to breathe can feel controlling, and focusing on the breath can at first increase anxiety. So we begin softly:

- Invite the client to notice the breath without changing it.
- Offer choices: eyes open or closed, sitting or lying, a hand on the belly or not.
- Use the language of invitation: "You might notice...", "If it feels okay..."
- Suggest a slightly longer exhale only once the client feels comfortable.
- Normalise that the breath may feel strange, tight or emotional, and that they can stop at any time.

Gentle regulating practices

These practices are generally suitable for most people and can be taught to clients for self-care. Keep them comfortable: comfort matters more than counting.

TRY THIS

The long, slow exhale

1. Sit or lie comfortably. Breathe in gently through your nose for a count of about four.
2. Breathe out slowly through your nose or softly pursed lips for a count of about six.
3. Continue for two to five minutes. If counting feels stressful, simply let each exhale be a little longer than the inhale.

TRY THIS

The cyclic sigh

1. Breathe in through your nose. At the top, take a second, shorter sip of air.
2. Then breathe out slowly and fully through your mouth, like a long sigh.
3. Repeat for one to five minutes, then return to natural breathing and notice how you feel.

TRY THIS **Voo breathing**

1. Peter Levine teaches a simple sound practice: after a comfortable inhale, breathe out with a long, low "voooo" sound, letting it vibrate in your chest and belly.
2. At the end of the sound, pause and let the next breath come by itself. Repeat three to five times.
3. Many people find the vibration grounding and calming. If sound feels exposing, try a soft hum instead.

Connected and intense breathwork: power and caution

Some breathwork methods use faster, fuller or continuous "connected" breathing for longer periods. Researchers call these high-ventilation practices. They can lead to very intense experiences: strong emotions, vivid memories or images, spontaneous movement, tingling and altered states of consciousness. I have seen them bring profound release, and I teach connected breathwork within a structured, trauma-informed training. But I want to be very clear about the risks.

SAFETY **intense breathwork is not for everyone**

Physical effects: breathing faster or deeper than the body needs lowers carbon dioxide in the blood. This can cause dizziness, light-headedness, tingling in the hands, feet and around the mouth, cramping or stiffness in the hands, chest tightness and a racing heart. These effects usually pass when breathing returns to normal, but they can be frightening and can feel like panic.

Emotional effects: intense breathwork can bring up overwhelming material quickly, push a person far above or below the window of tolerance, and trigger flashbacks or dissociation, especially in people with significant trauma.

Screening is essential: high-ventilation breathwork is generally not advised, or only with medical clearance, for people who are pregnant, or who have heart disease or high blood pressure, epilepsy or a history of seizures, glaucoma or retinal problems, recent surgery or injury, severe asthma, or a history of psychosis, bipolar disorder, severe dissociation or other serious mental illness. Follow the contraindications of your training method and ask a doctor when in doubt.

Never practise intense breathwork in or near water, while driving, or alone if you are new to it.

As a student: do not facilitate connected or intense breathwork with clients until you have been trained and supervised in that specific method.

A trauma-informed way of using breath

In my experience, the safest and most transformative breath work with trauma follows a simple rhythm: **resource first, charge gently, then rest**. We build safety and presence; we invite a little more breath and aliveness, in small doses; and then we let the breath return to normal and allow time for integration. If a

client becomes overwhelmed, we slow the breath down rather than pushing through. If they begin to disappear, we bring in eyes-open orienting, sound and gentle movement. The goal is never the biggest possible release.

> **The goal is never the biggest release. The goal is a nervous system that learns it can feel and still be safe.**

REFLECTION

- How does your breath change when you feel unsafe, rushed or judged?
- What is your own experience of intense breathwork? What made it feel safe or unsafe?

KEY TAKEAWAYS

- Begin with breath awareness and choice, not control.
- Slow breathing and longer exhales are supported by research as calming practices.
- High-ventilation breathwork can be powerful and has real physical and emotional risks. Screen carefully and practise only within your training.
- Resource, charge gently, rest: dose the breath to stay within the window.



CHAPTER 7

Combining Touch and Breath in a Session

A practitioner's guide to weaving the two doorways together

> Touch tells the body it is not alone. Breath tells it that it can move again.

In my sessions, touch and breath are rarely separate. Touch can guide the breath to places it has forgotten, and breath can help tissue release under the hands. Together, they form a conversation with the nervous system. This chapter is written for you as a practitioner: it offers a structure and principles for combining the two safely. Adapt it to your training and scope of practice.

The arc of a trauma-informed session

1. **Arrive**: welcome, intake and consent. Check current health, medication, recent life events, and what the client would like from the session. Agree on stop signals and how you will check in. Explain what you will do with touch and breath.
2. **Resource**: help the client feel what is supportive right now: the table beneath them, the warmth of a hand, a pleasant place in the body, a calm memory. Begin with natural breath awareness and a still, resourcing touch.
3. **Explore**: work with tension and breath in small doses (titration), moving between more difficult areas and resourced areas (pendulation). Check in often.
4. **Release**: if sensations or emotions arise, support them gently: slow the touch, breathe together, make space for tears, sighs, trembling or movement, and keep the client connected to the present.
5. **Integrate**: return to slow, natural breathing, stillness and resourcing touch. Allow time for rest. Silence is often the most important part of a session.

6 **Close**: bring the client back gently: eyes open, orienting to the room, sitting up slowly, a sip of water, a short conversation, and aftercare guidance.



The arc of a trauma-informed session. Each phase builds the safety needed for the next.

How touch supports the breath

- **Hand on the belly or lower ribs**: with consent, a still, warm hand can invite the breath to move lower. Suggest "breathing into my hand" only if that feels comfortable, or simply let the client notice the contact.
- **Hand on the upper back**: many people find support at the back of the heart less exposing than touch on the front of the chest.
- **Gentle side-rib contact**: two hands resting on the sides of the ribcage can encourage a wider, three-dimensional breath.
- **Following the breath**: let your hands move with the client's breath rather than imposing a rhythm.

How breath supports touch

- **Exhale into contact**: invite the client to breathe out slowly as you meet a tight area, and to rest at the end of the exhale while you wait at the edge.
- **Pause together**: before moving deeper, wait for one or two easy breaths. The breath will tell you whether the tissue and the nervous system are ready.
- **Breath as feedback**: if the breath is held or becomes high and fast, reduce pressure or move to a resource. If it deepens and slows, you are probably working at a good pace.
- **Your own breath**: breathe low and slow yourself. Clients often attune to the practitioner's breath without realising it.

Choosing the dose

Gentle and regulating	Activating: use with care and training
Natural breath awareness	Faster or fuller connected breathing
A long, slow exhale	Prolonged breath holds
Still, resourcing touch	Deep or sustained pressure near vulnerable areas
Eyes open; sitting or side-lying	Eyes closed; lying face down; strong music
Short sessions with frequent check-ins	Long sessions focused on emotional release

With trauma, start on the left side of this table. Move towards the right only with good resourcing, a clear agreement with the client, and training in the specific method. If in doubt, choose less.

> **If in doubt, choose less. You can always add more; you cannot take overwhelm back.**

Checking in without interrupting

Check-ins keep the client in charge. They can be very simple: "How is this pressure, on a scale from one to ten?" "What do you notice now?" "Would you like me to stay here, move on, or stop?" Agree on a non-verbal signal too, such as raising a hand, because some people lose access to words when they are activated.

TRY THIS **A 20-minute practice sequence for students**

1. Work with a practice partner from your training who has agreed to explore this.
2. Two minutes: arrive, agree on signals, and ask where a resourcing touch would feel good.
3. Three minutes: resourcing touch on the upper back or feet, with natural breath awareness.
4. Ten minutes: slow work on an area your partner chooses (for example the shoulders), with an exhale into contact and a pause at each edge. Check in every few minutes.
5. Three minutes: return to resourcing touch and natural breath, in silence.
6. Two minutes: eyes open, orient, sit up slowly, and share: what helped you feel safe?

REFLECTION

? Do you tend to lead with touch or with breath? What would change if you balanced the two?

? Which phrases help you check in naturally, without breaking the flow of a session?

KEY TAKEAWAYS

- ✓ A trauma-informed session has an arc: arrive, resource, explore, release, integrate, close.
- ✓ Touch can guide the breath, and breath can guide the touch. The client's breath is your best feedback.
- ✓ Dose carefully. Start gently, and move towards activation only with training and consent.



PART THREE

Safety, Release and Integration

Scope of practice, overwhelm, integration and self-care



CHAPTER 8

Safety, Consent and Scope of Practice

The container that makes healing possible

> Safety is not the absence of feeling. It is the presence of a container strong enough to hold it.

Trauma-informed work stands or falls on safety. Without it, even the most skilful technique can re-traumatise. With it, very simple touch and breath can be deeply healing.

Consent is an ongoing conversation

Consent is not a form signed at the beginning. It is a living agreement that the client can change at any moment.

- **Informed**: explain what you plan to do, where you will touch, what the breath practice involves, and what might happen (for example, emotions or tingling).
- **Specific**: ask for consent area by area, especially for the face, throat, chest, belly, inner thighs, buttocks and pelvis.
- **Freely given**: make "no" as easy as "yes". "You can say no or change your mind at any time, and I will not be offended."
- **Ongoing**: check in during the session, not only at the start.
- **Reversible**: if a client says stop, stop immediately and calmly, without questions or disappointment.

Many trauma survivors have learned to please, to freeze, or to say yes when they mean no. Watch for non-verbal signs too: if the body says no, treat it as a no, and check in.

Boundaries and the therapeutic relationship

Clear boundaries are a form of care. Keep the setting professional and predictable: clear time frames, appropriate draping, no sexual contact or comments, and no dual relationships that cloud your role. Be aware of **transference**, when a client unconsciously relates to you as they did to an important figure from their past, for example with idealisation, dependence or anger. Be aware of **countertransference** too: your own reactions, wishes to rescue, or old wounds that the session touches in you. These are normal human dynamics. Bring them to supervision rather than acting on them.

Scope of practice: bodywork is not psychotherapy

This may be the most important page in this ebook. As bodywork and breathwork practitioners, we can offer something precious: safety in the body, presence, and a space where held tension can soften. But we are not psychotherapists, unless we are separately trained and qualified as such. That means:

- We do not diagnose trauma, PTSD or any other condition.
- We do not try to uncover or "recover" memories, and we never suggest to a client what may have happened to them.
- We do not interpret their emotions or their story for them.
- We do not promise healing, cures or specific outcomes.
- We work alongside medical and psychological care, never instead of it.

SAFETY

When to refer

Refer the client to their doctor or a qualified mental health professional, and pause or adapt bodywork and breathwork, if they:

have thoughts of suicide or self-harm (in an emergency, call 112 or your local emergency number);

are in a current crisis, or are living in an unsafe or abusive situation;

regularly dissociate, have flashbacks or panic in or between sessions;

show signs of severe depression, psychosis, mania or an eating disorder;

are dealing with substance dependence;

disclose a recent trauma or abuse and seem to need more support than a session can offer;

have new, severe or unexplained physical symptoms, such as chest pain, numbness, weakness, fever or unexplained weight loss.

Have a list of local trauma-informed therapists, crisis lines and medical services ready before you need it.

Contraindications and medical caution

Always take a health history. Beyond the breathwork cautions in Chapter 6, deep tissue work may need to be avoided or adapted in cases such as a suspected blood clot or a history of deep vein thrombosis; recent injury, surgery or fracture; acute inflammation, fever or infection; certain skin conditions; uncontrolled high blood pressure; osteoporosis; bleeding disorders or blood-thinning medication; cancer and its treatment; and pregnancy. Ask for medical advice when in doubt, and follow the guidance of your training.

Trauma-informed language

Words can open or close the nervous system. Some simple shifts:

- Instead of "Relax", try "Let yourself be supported by the table."
- Instead of "Breathe deeper", try "You might notice your breath. If it feels okay, let the exhale be a little longer."
- Instead of "What happened to you?", try "What do you notice right now?"
- Instead of "Push through it", try "Let's slow down and find what feels manageable."
- Instead of "Great release!", try "Thank you for letting me know. How are you now?"

REFLECTION

? How do you make "no" easy for your clients?

? What is your plan if a client discloses something you are not trained to hold?

? Who is on your referral list?

KEY TAKEAWAYS

- ✓ Consent is informed, specific, freely given, ongoing and reversible.
- ✓ Bodywork and breathwork are not psychotherapy. We do not diagnose, interpret or promise.
- ✓ Know your red flags and your referral network before you need them.



CHAPTER 9

When a Client Is Overwhelmed or Dissociates

A calm, practical protocol for the moments that matter most

> Your calm is the first medicine in the room.

Sooner or later, a client will become overwhelmed in your session: panic, crying that seems to spiral, shaking that frightens them, a flashback, or a quiet disappearance into dissociation. These moments are not failures. They are the moments when your preparation and presence matter most.

First, regulate yourself

Feel your feet. Lengthen your own exhale. Soften your face and voice. The client's nervous system will be reading yours. You do not need to fix anything. You need to be steady.

A step-by-step protocol

1. **Pause the technique**: stop intense breathing and deep pressure. Keep or offer a simple, still contact only if the client wants it; otherwise take your hands away and stay close.
2. **Speak calmly and simply**: use their name. "You are here with me, in the studio. You are safe right now."
3. **Slow the breath, don't control it**: invite a longer exhale, or breathe slowly and audibly yourself so they can follow. Avoid asking for big inhales, which can add to dizziness.

4. **Orient to the present**: "Can you open your eyes? Let them look around the room. What can you see?" Name colours, objects and light.
5. **Bring in the senses**: feet pressing into the table or floor, hands feeling the blanket, a sip of water, a cool cloth, a sound.
6. **Offer movement and choice**: "Would you like to sit up? Turn on your side? Press your feet into my hands?" Gentle pushing with the hands or feet can help mobilise the body and bring a person back.
7. **Give time**: stay with them until their breath, colour and responsiveness return. Do not rush back into the session.
8. **Close and follow up**: talk about what happened in simple, non-interpretive words, make sure they are well enough to go home, and agree on a check-in.

TRY THIS **5-4-3-2-1 grounding**

1. Gently guide the person to name five things they can see, four things they can feel or touch, three sounds they can hear, two things they can smell, and one thing they can taste, or one kind thing they can say to themselves.
2. Practise it on yourself first, so you can offer it naturally and calmly.

If a client dissociates

Signs may include a blank stare, no response or delayed responses, very shallow breathing, saying they feel far away or unreal, or not remembering part of the session. Gently bring them back: say their name, ask them to open their eyes and look around, invite small movements of the fingers and toes, ask them to feel the weight of their body, and have them sit up slowly when ready. Speak in short, simple sentences. Afterwards, adapt future sessions: eyes open, more resourcing, shorter work, less activating breath and more check-ins.

If the person may be having a panic attack

A panic attack can feel like dying, but on its own it is not dangerous. Stay with the person, speak calmly, slow the exhale, and orient. However, chest pain, especially with breathlessness, sweating, or pain spreading to the arm, jaw or back, can also be a sign of a heart attack. If you are not sure, or if symptoms do not settle, call emergency services. Never assume it is "just panic".

SAFETY **After the session**

Write brief, factual notes about what happened and what you did.

Follow up with a message or call the next day.

If the client might be at risk, or the overwhelm is part of a pattern, recommend support from a qualified mental health professional.

Debrief with your supervisor or teacher. You need support after intense moments too.

KEY TAKEAWAYS

- ✓ Regulate yourself first; your calm is contagious.
- ✓ Pause, slow the breath, orient, use the senses, offer movement and choice, give time.
- ✓ Dissociation needs gentle reconnection and an adapted plan for future sessions.
- ✓ When symptoms could be medical, seek help. Never assume.



CHAPTER 10

Working with Release and Integration

Beyond catharsis: resourcing, titration, pendulation, completion and aftercare

> Healing is not a single breakthrough. It is a spiral, where each turn brings a little more space.

There was a time when trauma work was mostly about catharsis: screaming, pounding, pushing through. I have walked that path myself, as a client and as a facilitator. It has its place, and sometimes it brings great relief. But I have also seen people leave intense sessions more dysregulated than when they arrived. Over the years my work has become gentler and slower, and I believe it has also become deeper.

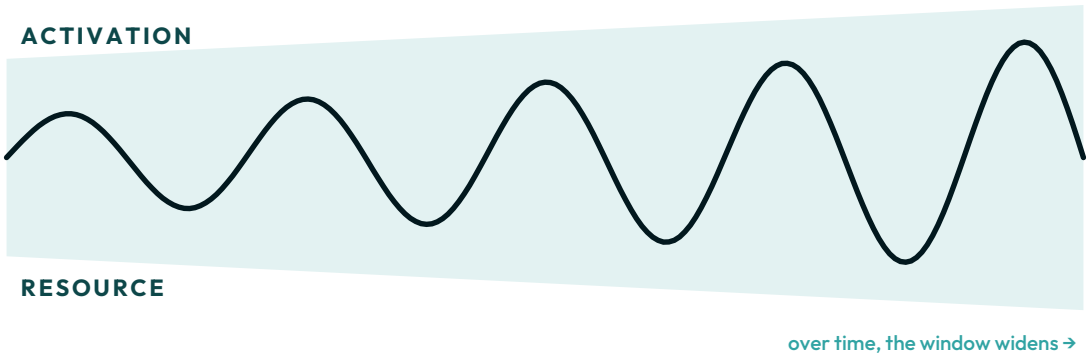
> **“ Gentleness is not weakness in trauma work. It is its foundation.**

Four keys to gentle release

These ideas come from somatic trauma therapy, especially Peter Levine's Somatic Experiencing, and have been widely adopted by trauma-informed bodyworkers and breathworkers:

- **Resourcing**: helping the client connect with what supports them: a safe place in the body, a calm memory, a person, the ground. Resources come first and are returned to often.
- **Titration**: working with small, manageable doses of difficult sensation or emotion, adding a few drops at a time rather than pouring.
- **Pendulation**: moving gently back and forth between a more difficult area or feeling and a resourced, comfortable one, so the system learns that it can touch distress and come back.

- **Completion**: allowing protective impulses that were interrupted, such as pushing away, turning, running or curling up, to be gently felt and expressed, often in slow motion, so that the body can finish what it started.



Pendulation: moving between resource and activation in small doses, so that the window of tolerance can widen.

What release may look like

When a nervous system lets go, you may see trembling or shaking, waves of heat or cold, tears, deep sighs, yawning, laughter, gurgling in the belly, spontaneous stretching or movement, tingling, or a sudden stillness and peace. Many practitioners observe that these responses tend to arise naturally when a person feels safe enough, and that they are often followed by a sense of lightness and ease.

Our role is to allow, not to provoke. Support release by staying present, slowing down, keeping the client oriented, and making sure they stay within their window. If release becomes a spiral, we come back to resources. We do not need to understand or analyse what is coming up. As I often tell my students: feeling is healing, but only when it is felt in safety.

Unwinding and spontaneous movement

Sometimes the body wants to move: a slow curling, a turn of the head, a pushing of the legs. When we gently follow these movements without leading them, many clients report a deep sense of release and integration. Keep movement slow and supported, protect the joints and the neck, and stop if the movement becomes frantic or the client seems to disappear.

Integration: where the change settles

Integration is the most underestimated part of trauma work. After release, the nervous system needs time to reorganise. Give it:

- **Stillness**: several minutes of silence and resourcing touch at the end.
- **Grounding**: feet, breath, eyes open, sitting up slowly.
- **Gentle reflection**: "What do you notice now?" rather than analysing the story.

- **Aftercare guidance**: drink water, eat something, rest, walk in nature, avoid big decisions or intense activities for the rest of the day, and journal if that helps.
- **Normalising after-effects**: some people feel tired or emotional, or have vivid dreams, for a day or two. Invite them to contact you if anything feels too much, and to contact their doctor or therapist if needed.

TRY THIS

Completing a protective movement

1. Sit comfortably and recall a mildly stressful situation, nothing overwhelming, in which you felt you could not say no.
2. Notice where your body tenses. Now, very slowly, press your palms forward as if gently setting a boundary.
3. Feel the strength in your arms and back. Breathe out. Then let your hands rest and notice what has changed.
4. If anything feels like too much, open your eyes, look around, and feel your feet.

REFLECTION

- ? What have you learned from your own experiences of release, gentle or intense?
- ? How much time do you currently give to integration in your sessions?

KEY TAKEAWAYS

- ✓ Release is allowed, never provoked. Big catharsis is not the goal.
- ✓ Resourcing, titration, pendulation and completion keep the work within the window.
- ✓ Integration needs time, stillness, grounding and clear aftercare.



CHAPTER 11

Self-Care for the Practitioner

You cannot pour from an empty body

> You cannot teach others something you are not.

When we hold space for other people's pain, some of it touches us. This is not a weakness. It is a sign that we are human and open. But without care, it can become a burden.

Vicarious trauma and compassion fatigue

Vicarious trauma is a gradual change in our own inner world through repeated exposure to the trauma of others: we may start to feel unsafe or cynical, or be haunted by clients' stories. **Compassion fatigue**, a term popularised by the trauma researcher Charles Figley, describes the emotional and physical exhaustion that can come from caring for people in distress without enough recovery. Signs may include tiredness, irritability, numbness, sleep problems, dreading sessions, losing joy in the work, and pulling away from loved ones.

My practices for staying resourced

- **Arrive and leave consciously**: a short ritual before and after each session. I wash my hands slowly, feel my feet, and take three long exhales, letting the last session go before the next one begins.
- **Move every day**: walking, qigong, yoga, dancing or swimming help my body discharge what it has held.
- **Nature as medicine**: time among trees, near water, or with bare feet on the earth is one of my most reliable ways to come back to myself.
- **Receive, not only give**: I regularly receive bodywork and breathwork sessions myself.

- **Supervision and community**: regular supervision, peer groups and my own therapy keep me honest and supported. No one should do trauma work alone.
- **Boundaries with my time**: a sustainable number of sessions, real breaks and holidays. Rest is not a reward for exhaustion; it is a need.
- **From therapist mode to me mode**: laughing with friends, intimacy, play and creativity remind me that I am more than my role.

TRY THIS

The two-minute session reset

1. Between clients, stand with your feet hip-width apart. Gently shake your hands, arms and legs for 30 seconds.
2. Then stand still. Take three slow breaths with long exhales. Place one hand on your heart and silently say: "I let go of what is not mine."
3. Wash your hands slowly in cool water and feel the water on your skin.

REFLECTION

- ? Where in your body do you notice the impact of your work?
- ? What would a sustainable week of practice look like for you?
- ? Who supports you, and when did you last ask for help?

KEY TAKEAWAYS

- ✓ Vicarious trauma and compassion fatigue are common in the helping professions. Recognise the signs early.
- ✓ Self-care is a professional responsibility, not a luxury.
- ✓ Supervision, community and receiving sessions yourself are essential.



CLOSING

The Body Whispers: An Invitation

We have travelled a long way together in these pages: from what trauma is, through the fascia and the nervous system, to the two doorways of touch and breath, and finally to safety, release, integration and care.

If there is one thing I hope you take with you, it is this: the body whispers. Long before it shouts in pain or illness, it speaks in subtle ways, through a held breath, a tight jaw, a hand that cannot rest. Our work as bodyworkers and breathworkers is to slow down enough to listen, and to create a space safe enough for the body to tell its story at its own pace.

You do not need to be perfect to do this work. You need to be present, humble, well trained and well supported. The rest, very often, the body knows.

Continue your journey with me

Reading about trauma-informed work is a beginning. Real skill is learned in the body: in practice, with feedback, supervision and community. I warmly invite you to continue your journey in one of my trainings or retreats, where we explore trauma-informed bodywork and breathwork in depth, step by step, in a safe and loving space.

Integral Body Institute: professional trainings in bodywork and breathwork, in Poland and internationally. **integralbodyinstitute.com**

Nisarga8: private online sessions, retreats, my calendar and free resources. **nisarga8.com**

I look forward to meeting you at the table, on the mat, and in the breath.

With gratitude,

Nisarga



THE AUTHOR

About Nisarga Eryk Dobosz

Nisarga Eryk Dobosz is a faculty member of Myofascial Energetic Release (MER®) and Biodynamic Breathwork and Trauma Release (BBTRS®). He began his bodywork journey in 1998, drawn by a deep passion for connecting with people through touch. He sees each person as a whole of body, mind and soul, so his work reaches beyond the physical to the emotional and spiritual layers where pain and tension are held.

Alongside MER® and BBTRS®, he has trained in Compassionate Inquiry (CI®), the NeuroAffective Relational Model (NARM®) and Biodynamic Craniosacral Therapy (BCST®). He combines bodywork, breathwork and movement to help people release pain and tension, and he trusts the body's innate capacity to regulate and heal itself.

Nisarga teaches and facilitates groups internationally. His purpose is to celebrate the resilience of the human body and to guide people back to a life of vitality, ease and connection with body, mind and soul.

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